DART

DETERIORATION AND RESUSCITATION TEAM

Introduction

- Who we are and what do we do
- Common themes
- How can we improve our practice?

DH(DUHNFT1

Who are we?

- New name!
- 08:00-18:30 7 days a week
- Resuscitation training
- #007 - Work clinically to help identify and manage acute deterioration
- support the escalation to medics/surgeons or consider limits of care where necessary
- We aim to help develop the skills of all staff through support, reflection, and local education
- Attend all cardiac arrest and trauma calls within working hours
- Audit – #007, NCAA, NEWS2, emergency equipment

Slide 3

DH(DUHNFT1 DICKSON, Helen (ROYAL DEVON UNIVERSITY HEALTHCARE NHS FOUNDATION TRUST), 10/09/2023

Common themes

Escalation

TEP

Response

Identifying dying

Escalation

Non-compliance occurs at all levels:

- taking obs (70%)
- interpreting obs
- Escalation
- response

Physiological parameter	Score 3	2	1	0	1	2	3
Respiration rate (per minute)	≤8		9–11	12–20		21–24	≥25
SpO_2 Scale 1 (%)	≤91	92–93	94–95	≥96			
SpO_2 Scale 2 (%)	≤83	84–85	86–87	88–92 ≥93 on air	93–94 on oxygen	95–96 on oxygen	≥97 on oxygen
Air or oxygen?		Oxygen		Air			
Systolic blood pressure (mmHg)	≤90	91–100	101–110	111–219			≥220
Pulse (per minute)	≤40		41–50	51–90	91–110	111–130	≥131
Consciousness				Alert			CVPU
Temperature (°C)	≤35.0		35.1–36.0	36.1–38.0	38.1–39.0	≥39.1	

At all times consider conversations and use of a Treatment Escalation Plan		
Stable 0	Low Score Group	Normal Observations minimal 12 hourly: Respirations; Pulse; BP; Oxygen Sats; Temperature; AVPU
Low Risk 1-4	Potential for Deterioration	Inform Nurse in Charge who must assess patient and decide on monitoring plan or if escalation required Observations at least 4 hourly (more frequent could be required)
3 in a single parameter	Deteriorating Patient	Observations at least 2 hourly (more frequent could be required) Urgent Doctor to review who must define medical plan. Consider bleep to outreach team on 007. If unable to improve vital signs in one hour or serious concern call a more senior doctor
Medium Risk 5- 6 or nurse concern	Deteriorating Patient	Observations at least 2 hourly (more frequent could be required) • Ensure urine output is monitored and recorded Urgent Doctor to review who must define medical plan. Consider bleep to outreach team on 007. If unable to improve vital signs in one hour or serious concern call a more senior doctor.
High Risk 7 plus	Critically Ill Patient	Observations at least hourly (more frequent could be required) Urgent Dr (at least F2 or more senior) to review. A Medical Plan must be defined. Following initial review to be discussed with senior colleague at earliest opportunity. If unable to improve in one hour or serious concern call a more senior doctor / Consultant Consider bleep to outreach team on 007.
Patient at Ceiling of Care	On Ward Based Care	4-8 hourly observations with a defined medical plan
Patient Imminent chance of Cardio-Respiratory Arrest	**Call the Arrest Team on 2222** **If patient has a DNAR order then consider call to family and on call doctor**	

NHS England

What are we doing:

- Feeding back audit results
- Training
- Support (007)
- Wider MDT

Response to escalation

- Nights & weekends - On-call teams who don't know the patients
- Disparity in documented plans
- Task allocation ahead of time – bloods, W/E review, IVI
- Good concise Friday summary – clear directive for patient care for non-parent teams to follow confidently out of hours
- Scared to make the decision to do nothing

TEP

- August – 34/68 (50%) of patients we were involved in had TEPS already. We identified a further 7 that need one.
- Early discussions
- Forward planning
- Leaflets

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Decisions about Cardiopulmonary Resuscitation (CPR)

Other formats

If you need this information in another format such as audio CD, Braille, large print, high contrast, British Sign Language or translated into another language, please contact the PALS desk on 01271 314090 or at ndht.pals@nhs.net.

Introduction

This leaflet is about Cardiopulmonary Resuscitation (CPR) and how decisions are made about it. This leaflet may not answer all of your questions, so please talk to a member of the healthcare team looking after you if there is anything that you do not understand, or if you would like more information

What is Cardiopulmonary Resuscitation (CPR)?

CPR is a treatment that is used to try to restart someone's heart and breathing, when one or both of these has stopped. When the heart stops this is called a cardiac arrest and when the breathing stops it is a respiratory arrest. If one stops, the other will usually stop soon afterwards (cardiopulmonary arrest).

CPR includes:

- Repeatedly pushing down very vigorously on the centre of the chest;
- Blowing air or oxygen into the lungs, using either a mask over the nose and mouth or a tube inserted into the throat or windpipe; and often
- Using electric shocks to try to restart normal heartbeats.

What is the chance of CPR restarting my heart and breathing?

There is no simple answer to this, because everyone is different. The chance of CPR restarting your heart and breathing will depend on:

Identifying death

- #007 called to wards regularly
- Identifying the dying phase too late
- Patients unable to express wishes
- Difficult to know when to stop – importance of early discussions and understanding patient's wishes
- 18% of patients we have seen this year were made COC, EOL or died whilst on our caseload.

What can we do?

- **We want to establish collaborative working and improve communication across the MDT in order to ensure better management of the deteriorating patient.**

- Improve escalation
- Early TEP discussions
- Continuity of plans, especially over weekends
- Accessible clinicians
- Realistic planning (COC, patient wishes ect)

Thank you!